HARBOURSIDE
CARDIOLOGY

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3 November 2026

Dr James Smith
Sydney Aged Care

Re: Jane Doe | DOB 1 November 1935 | JD-19351101

Sydney Aged Care

Dear Dr Smith,

Telephone case consultation: dizziness and recent results

Thank you for asking me to review Jane's ongoing dizziness in the context of atrial fibrillation and her recent blood results. I undertook a telephone case consultation with Morgan Bell, the facility registered nurse, today. This letter follows my September advice and the completed GP consultation of 21 October. Jane was not examined during this case consultation.

The supplied history describes intermittent dizziness on getting up, continuing in Jane's own update of 23 October. Emily observed Jane asking for support and pausing on their walk to the dining room on 24 October. Staff have subsequently recorded similar requests. Jane has also described remaining in her room for breakfast because she worries about becoming dizzy. These reports indicate a practical effect on participation and confidence; they do not establish a cardiovascular cause.

Her usual mobility is with a frame, and she normally eats independently. Atrial fibrillation was documented in May 2025, with hypertension and knee osteoarthritis also recorded. No current examination, standing blood-pressure series, pulse record or ECG accompanies this referral. I have not inferred these findings from her medication chart.

Medicine	Directions in the 28 August 2026 chart
Apixaban	5 mg twice daily
Metoprolol	25 mg twice daily
Amlodipine	2.5 mg once daily
Rosuvastatin	5 mg at night

The chart records a reduction of amlodipine from 5 mg to 2.5 mg on 28 August. No subsequent issued dose change is documented. Jane's question about which heart tablet changed remains relevant to reconciliation and to a clear explanation of her current regimen. This correspondence does not issue or renew a prescription.

The full blood examination collected on 27 October reports haemoglobin 101 g/L (laboratory interval 115–165), compared with 128 g/L on 14 April. The 29 October electrolyte panel reports sodium 122 mmol/L (135–145), previously 138 mmol/L, with an explicit laboratory critical flag. Potassium is 4.3 mmol/L (3.5–5.2) and creatinine 74 µmol/L (45–90). The reports were issued after the earlier September correspondence. There is no documented clinical assessment of these new abnormalities in the material provided.

My impression is persistent positional dizziness with reduced confidence in walking and communal meal attendance, of undetermined cause. The low sodium and lower haemoglobin warrant prompt medical assessment in their own right. It would be premature to attribute Jane's symptoms to either result, to atrial fibrillation or to a particular medicine without examination and reconciliation.

Recommendations

1. Please arrange same-day in-person medical assessment of Jane and review the laboratory critical alert. The assessing team should determine the appropriate setting and repeat blood tests; a follow-up appointment alone should not substitute for this assessment.

2. Reconcile the medication administration record with the current chart, including the amlodipine reduction. Review pulse, lying/standing blood pressure where safe, current weight and renal function when assessing treatment. No empiric dose change is issued in this letter.

3. Assess the fall in haemoglobin and ask about possible bleeding, particularly while Jane is prescribed apixaban. Decisions about anticoagulation should follow the treating team's assessment; INR is not being requested as routine monitoring of apixaban.

4. Please explain the recorded medication change to Jane in plain language and provide a written list, involving Emily with Jane's permission. Arrange cardiology follow-up after the immediate assessment, with earlier review if symptoms persist or the assessment identifies a cardiovascular concern.

I would appreciate the assessment findings and updated medication list so that the next cardiology review can address the outstanding symptoms with a reconciled record.

Yours sincerely,

Dr Matthew Thompson
Cardiologist